

Context-Dependent Relational Reorganization of Heart Rate Dynamics Precedes Spontaneous Ventricular Fibrillation: Systemic Tau and Ordinal RECD Evidence from the Sudden Cardiac Death Holter Database

Johel Padilla-Villanueva, DrPH

University of Puerto Rico, Medical Sciences Campus, San Juan, Puerto Rico

ORCID: 0000-0002-5797-6931 · github.com/johelpadilla/systemictau

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Abstract

Sudden cardiac death from ventricular fibrillation (VF) remains difficult to anticipate from surface electrocardiography. Classic early-warning signals (EWS) such as rising variance or lag-1 autocorrelation often fail or reverse in real cardiac data because they assume a simple critical-slowness scenario. We introduce a relational, multivariate analysis pipeline that combines (i) **Systemic Tau** (τ_s), an ordinal, Kendall-based measure of cross-variable coupling change, with (ii) **ordinal recurrence quantification** under the Discrete Extramental Clock (RECD) framework, which decomposes a bivariate heart-rate proxy into hierarchical symbolic levels (Φ_1, Φ_2, Φ_3) and tracks the continuous excess contribution of the highest level (excess3).

Applied to $N = 10$ high-quality records from the PhysioNet Sudden Cardiac Death Holter Database (SDDB; 23 records total), we find that $\Delta\tau_s$ and $\Delta\text{excess3}$ are statistically extreme under phase-shuffle surrogates in the majority of cases and, crucially, maintain **sign concordance** (8/10 records) even when the direction is opposite to classical variance increase. Intermittent pacing and atrial fibrillation cases were retained after explicit quality flagging and did not abolish the signal. Two records with small effect sizes (47 and 50)

showed discordant direction and are interpreted as borderline transitions. Light re-calibration of synthetic-derived thresholds ($\theta_3 = 0.08$, high-threshold = 0.65, relative λ) is documented; the continuous excess3 metric remains primary.

These results support a view of pre-VF transitions as *context-dependent reorganizations of the relational structure of heart-rate dynamics*, rather than as a uniform loss of stability. The findings provide real-world physiological support for the Systemic Tau and RECD framework as tools capable of detecting relational reorganizations that precede critical transitions in noisy biological signals where classical univariate EWS fail or reverse. The pipeline is fully reproducible and immediately extensible to other Holter collections.

Keywords: Systemic Tau; Discrete Extramental Clock (RECD); ordinal patterns; early-warning signals; ventricular fibrillation; heart-rate variability; network physiology; PhysioNet SDDb; critical transitions; surrogate testing.

1. Introduction

1.1 Clinical and scientific motivation

Sudden cardiac death (SCD) is a leading cause of mortality worldwide, and ventricular fibrillation (VF) is among its principal terminal rhythms. Despite decades of progress in implantable devices, risk stratification, and Holter analysis, anticipating spontaneous VF from continuous surface ECG remains an open problem in both clinical cardiology and complex-systems physiology ([Goldberger et al. 2000](#); [Ivanov et al. 1999](#); [Bashan et al. 2012](#)).

Classical approaches emphasize univariate heart-rate variability (HRV) descriptors—variance, lag-1 autocorrelation, fractal scaling, multifractality—and, more recently, machine-learning classifiers trained on pre-VF segments from public databases such as the Sudden Cardiac Death Holter Database (SDDb) ([Greenwald 1986](#); [PhysioNet 2004](#); [Velázquez-González et al. 2021](#); [Heng et al. 2020](#)). These tools are valuable, yet they often inherit an implicit dynamical assumption: that the approach to VF resembles a generic critical transition marked by critical slowing down (CSD), with rising variance and rising autocorrelation ([Scheffer et al. 2009, 2012](#); [Dakos et al. 2012](#)).

Empirical cardiac dynamics frequently violate that assumption. Variance may rise while lag-1 autocorrelation falls; atrial fibrillation (AF) and intermittent pacing can dominate the signal; annotation quality varies; and the same numerical “early warning” can point in opposite directions depending on the clinical substrate. What is needed is a language that tracks *relational reorganization*—how the coupling among dynamical degrees of freedom changes—rather than only the amplitude of fluctuations in a single series.

1.2 A short primer on Systemic Tau for readers outside the paradigm

Systemic Tau (τ_s) is an ordinal, multivariate stability metric developed for noisy, non-stationary complex systems (Padilla-Villanueva 2025b, 2026a, 2026b). At its core it uses rank-based (Kendall-type) association structure across simultaneous series, computed in sliding windows, to quantify how *relational coherence* evolves in time. Three properties make it attractive for Holter RR analysis:

1. **Ordinal robustness.** Because ranks replace raw amplitudes, τ_s is comparatively robust to monotone rescaling and to moderate outliers—a practical advantage on interpolated RR series.
2. **Multivariate by construction.** Systemic Tau is not a univariate complexity score; it measures change in *cross-variable* organization. In this study the bivariate proxy is $X(t) = [z(\text{RR}), z(|\Delta\text{RR}|)]$, pairing level and successive irregularity.
3. **Early-warning orientation.** Large or systematic shifts in τ_s are interpreted as candidate signals of structural reorganization, not necessarily as “approaching collapse” in the classical CSD sense. The *sign* of $\Delta\tau_s$ carries information about the *direction* of the reorganization (e.g., tightening vs loosening of relational structure).

Readers familiar only with variance-based EWS may think of τ_s as answering a different question: not “Is the system fluctuating more?” but “Is the *pattern of mutual organization* among variables changing as the event approaches?”

1.3 Ordinal RECD levels and excess3

The Discrete Extramental Clock (RECD) framework extends Systemic Tau by building hierarchical ordinal conjunctions from Bandt–Pompe symbolic dynamics (Bandt and Pompe 2002; Padilla-Villanueva 2026b). On a multivariate series one forms short ordinal patterns (here embedding dimension $m = 3$, delay 1) and accumulates three nested levels of structure:

- Φ_1 : coincidence / local ordinal co-occurrence;
- Φ_2 : persistent pairwise relational structure;
- Φ_3 : higher-order conjunctions, with a continuous **excess3** term quantifying the surplus contribution of the highest level beyond lower-order expectations.

In practice, **mean excess3** (and its basal-to-approach delta) is the primary continuous readout. A binary high-level rate (fraction of windows with excess3 above a threshold) is retained for future extensibility but is secondary on noisy Holter data (see Section 3.3).

A weighted RECD variant modulates level contributions by a λ factor derived from $|\tau_s|$, coupling the two modules of the pipeline. When observed $|\tau_s|$ is small relative to synthetic design thresholds, λ is nearly constant and weighted results collapse toward the unweighted ones—an empirical finding we report transparently.

1.4 Contribution of this work

This article provides the first systematic application of Systemic Tau and ordinal RECD to spontaneous human pre-VF Holter dynamics, with:

1. a quality-aware $N = 10$ analysis of SDDDB under realistic inclusion constraints;
2. explicit retention and flagging of intermittent pacing and AF;
3. surrogate-based tests of non-trivial relational structure;
4. documented light re-calibration of thresholds derived from synthetic series;
5. full reproducibility commands and a clean figure set suitable for journal review.

The central empirical claim is modest but sharp: **pre-VF heart-rate dynamics exhibit context-dependent relational reorganization that is sign-concordant between τ_s and excess3**, even when classical univariate EWS are weak, reversed, or clinically confounded.

2. Data and Methods

2.1 Database and inclusion

We used the PhysioNet Sudden Cardiac Death Holter Database (SDDDB) ([PhysioNet 2004](#); [Greenwald 1986](#); [Goldberger et al. 2000](#)). SDDDB comprises exactly 23 complete Holter recordings (records 30–52), typically ~ 24 h, sampled at 250Hz with two ECG leads. The cohort includes 18 subjects with underlying sinus rhythm (four with intermittent pacing), one continuously paced subject, and four with atrial fibrillation. VF onset markers (`#vfon`) appear in header comments for VF cases.

After strict filters—usable pre-event duration, clear event anchoring, low invalid-RR / interpolation fraction, and automated pacing flagging—the realistic high-quality ceiling on SDDDB is approximately 8–12 records. Our final analytic set is $N = 10$: records **30, 31, 32, 35, 36, 38, 45, 47, 50, 51**. Intermittent pacing in 32 and 51 was detected and retained.

2.2 RR extraction and quality metadata

Beat annotations were read with WFDB (`wfdb.rdann`), preferring audited `.atr` files and falling back to unaudited `.ari` when necessary. RR intervals (ms) were obtained as successive sample differences scaled by sampling frequency. Intervals outside $(250, 2000)$ ms were treated as invalid and linearly interpolated. For each record we export: number of beats, interpolation fraction, coefficient of variation of RR, `pacing_detected`, and known pacing type (comment + known-record list + low-CV heuristic).

2.3 Bivariate proxy and Systemic Tau

Define the bivariate series

$$X(t) = [z(\text{RR}), z(|\Delta\text{RR}|)],$$

where $z(\cdot)$ denotes z-scoring within the analysis stream. Systemic Tau was computed with the published `systemictau` implementation (Padilla-Villanueva 2026b) using window $W_\tau = 101$ beats and stride 5. Basal and approach regimes were defined as record-specific ~ 3 h windows anchored to VF onset or end-of-recording (preserving legacy windows for records 30 and 35). The primary contrast is

$$\Delta\tau_s = \overline{\tau}_s^{(\text{approach})} - \overline{\tau}_s^{(\text{basal})}.$$

2.4 Surrogate testing

To test whether $\Delta\tau_s$ reflects genuine cross-dependence rather than marginal spectra, we generated phase-shuffle surrogates that approximately preserve individual power spectra while destroying cross-variable dependence (Theiler et al. 1992). For each record we used $n = 8$ independent surrogates and report the directional surrogate p -value (fraction of surrogates with $|\Delta\tau_s^{\text{surr}}|$ at least as extreme, with ties handled conservatively). Classical Welch tests on windowed samples are also reported for descriptive completeness.

2.5 Ordinal RECD levels and weighted RECD

Ordinal patterns used Bandt–Pompe encoding with $m = 3$, delay 1, window $w_\phi = 101$, stride 5 (Bandt and Pompe 2002). Nested levels Φ_1, Φ_2, Φ_3 and continuous excess3 were computed on the same $X(t)$ and the same basal/approach windows as τ_s . Primary metrics: mean excess3 and $\Delta\text{excess3}$.

Light re-calibration (motivated by observed RR statistics; see Section 3.3):

- θ_3 : $0.10 \rightarrow 0.08$;
- high-threshold for binary high-level rate: $1.75 \rightarrow 0.65$;
- λ : record-relative scaling $|\tau_s| / \max |\tau_s|$ (or lowered absolute chaos threshold), because observed $|\tau_s|$ on Holter RR is much smaller than synthetic design values (~ 0.41).

Weighted RECD completed on 6/10 records (30, 31, 32, 35, 38, 51); remaining records use unweighted excess3. A `has_weighted` flag is retained in the batch table.

2.6 Classical EWS for comparison

On the same RR series we computed rolling variance and lag-1 autocorrelation as standard CSD-style EWS (Scheffer et al. 2009; Dakos et al. 2012), reported as basal-to-approach deltas for qualitative comparison with relational metrics.

2.7 Software and reproducibility

Analyses used Python 3 with NumPy/SciPy/Pandas/Matplotlib, WFDB, and `systemictau` (Padilla-Villanueva 2026b). Batch orchestration, quality reporting, and figure generation are provided by project scripts (`run_cctp_batch.py`, `run_recd_on_rr.py`, `run_recd_weighted_on_rr.py`, `analyze_cctp_pilot.py`). Exact commands appear in Appendix A.

3. Results

3.1 Cohort summary

Table 1 reports per-record $\Delta\tau_s$, surrogate p , $\Delta\text{excess3}$, excess3 p -value, interpolation fraction, and pacing status for the $N = 10$ analytic cohort.

Table 1: Batch overview ($N = 10$). $\Delta\tau_s$ and $\Delta\text{excess3}$ are approach minus basal. Surrogate p from phase-shuffle tests on $\Delta\tau_s$ ($n = 8$). Interpolation fraction in percent. Pacing: automated flag.

Record	$\Delta\tau_s$	p_{surr}	$\Delta\text{excess3}$	p_{ex3}	Pacing / note
30	-0.0274	0.00	-0.0118	7.7×10^{-78}	none
31	-0.0001	0.25	-0.0221	1.1×10^{-222}	none
32	-0.0239	0.00	-0.0088	5.1×10^{-108}	intermittent
35	+0.0158	0.00	+0.0036	3.2×10^{-29}	none
36	+0.0786	0.00	+0.0080	1.0×10^{-17}	AF
38	+0.0876	0.00	+0.0414	≈ 0	none
45	+0.0066	0.25	+0.0043	0.003	none
47	-0.0026	0.25	+0.0300	4.3×10^{-68}	none
50	+0.0722	0.00	-0.0033	0.0009	AF
51	-0.0569	0.00	-0.0322	≈ 0	intermittent

Two records with small effect sizes (47 and 50) showed discordant direction between $\Delta\tau_s$ and $\Delta\text{excess3}$; these cases had among the smallest absolute relational deltas in the cohort and are interpreted as borderline transitions where reorganization is subtle.

Sign concordance between $\Delta\tau_s$ and $\Delta\text{excess3}$ holds in **8/10** records. Six records achieve surrogate $p = 0.00$ for $\Delta\tau_s$. Strongest absolute relational shifts include records 38 ($\Delta\tau_s = +0.0876$, $\Delta\text{excess3} = +0.0414$), 36 ($+0.0786$, $+0.0080$), 51 (-0.0569 , -0.0322), and 30 (-0.0274 , -0.0118). Direction is context-dependent: positive for several AF/terminal patterns and negative for several paced/intermediate patterns.

3.2 Case studies: strong positive and strong negative reorganization

Figure sets for individual records illustrate that relational metrics are not redundant with variance alone.

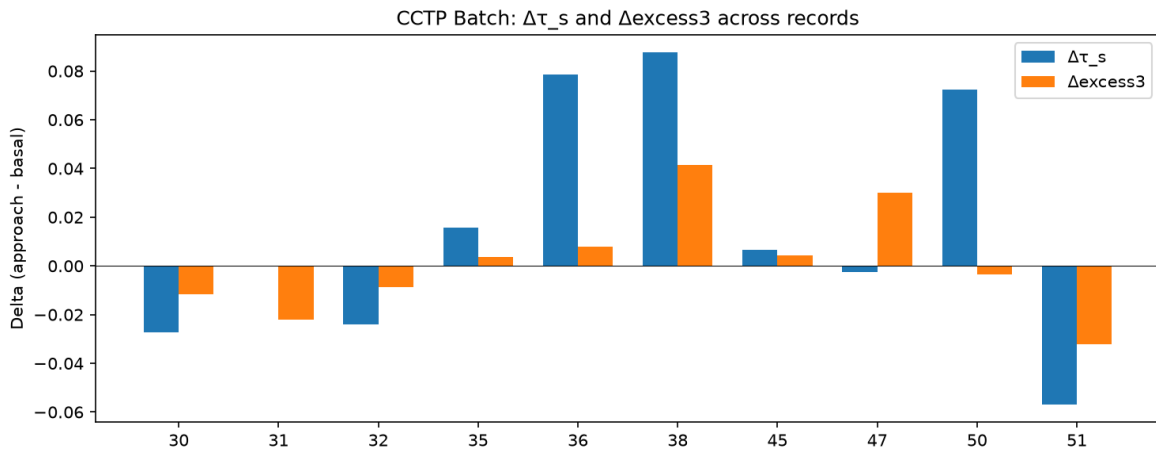


Figure 1: Batch comparison of $\Delta\tau_s$ and $\Delta\text{excess3}$ across the $N = 10$ cohort. Concordant signs dominate; absolute magnitudes vary by clinical context.

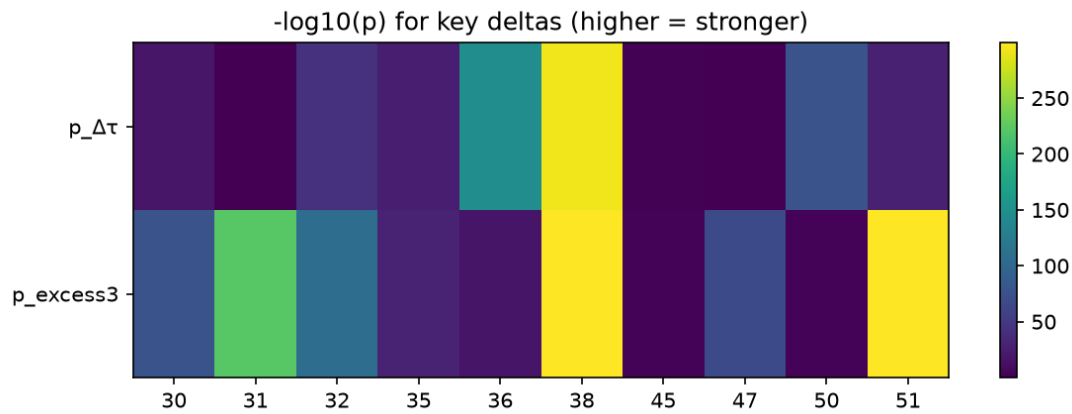


Figure 2: Significance overview ($-\log_{10} p$) for relational metrics across records.

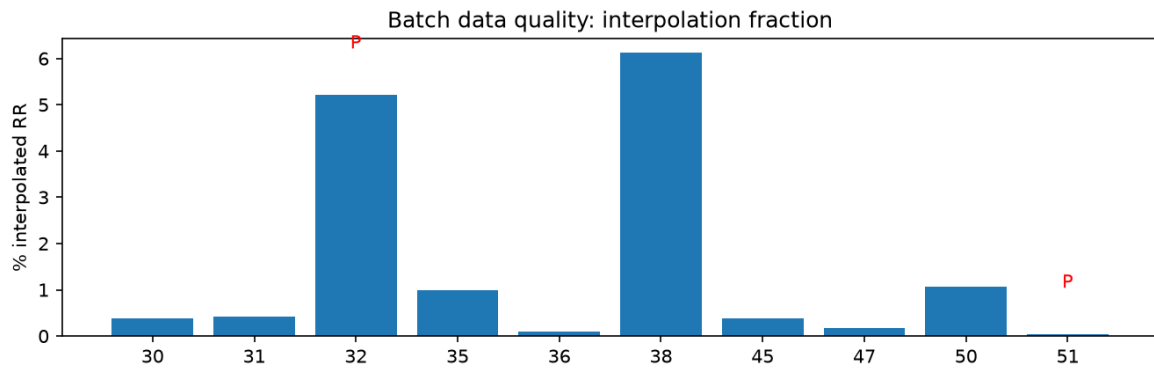


Figure 3: Data-quality summary: interpolation fraction and pacing flags (“P”). Record 32 has the highest paced-related interpolation burden among retained records but does not reverse cohort-level sign concordance.

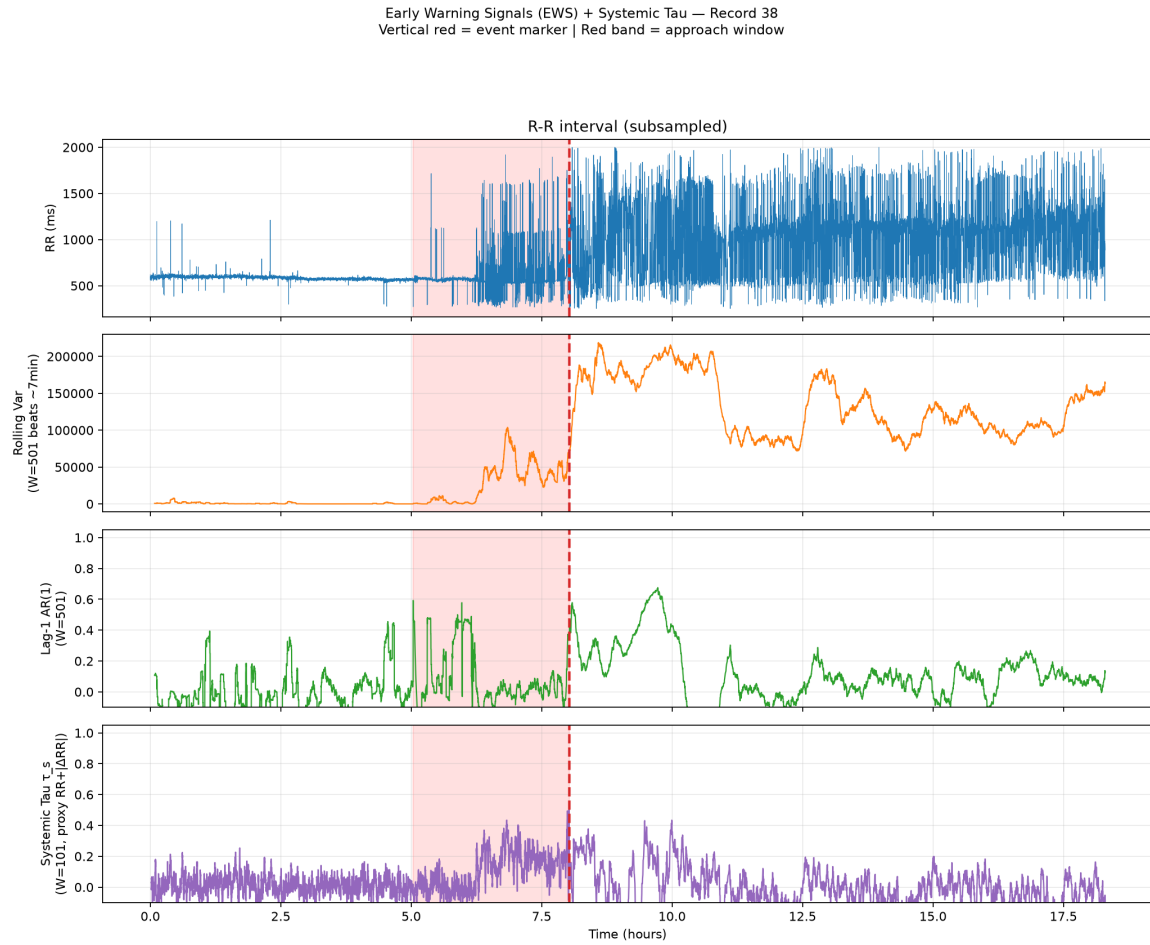


Figure 4: Early-warning panels for record 38: RR, variance, lag-1 autocorrelation, and Systemic Tau. Red vertical line marks the event anchor; shaded band marks the approach window.

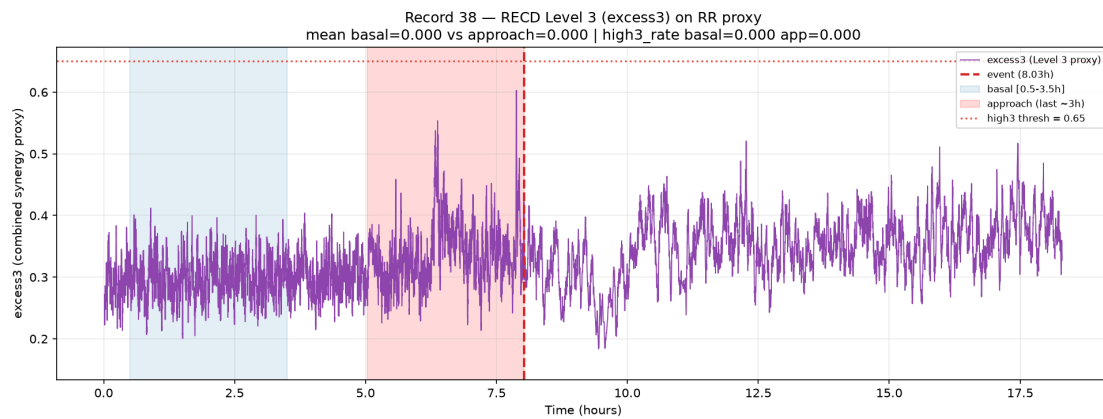


Figure 5: RECD excess3 time series for record 38 with basal and approach windows.

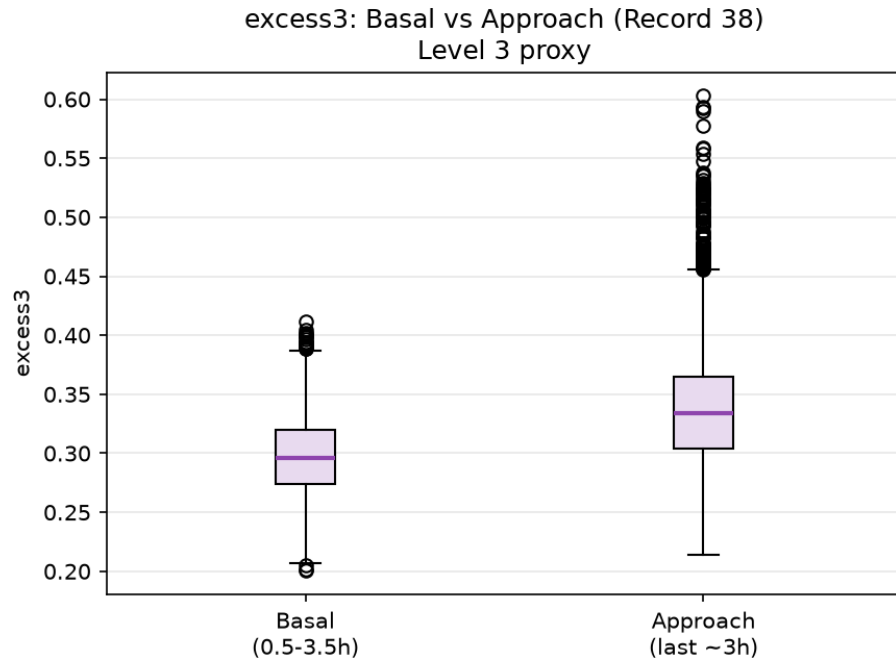


Figure 6: Basal vs approach distributions of excess3 for record 38.

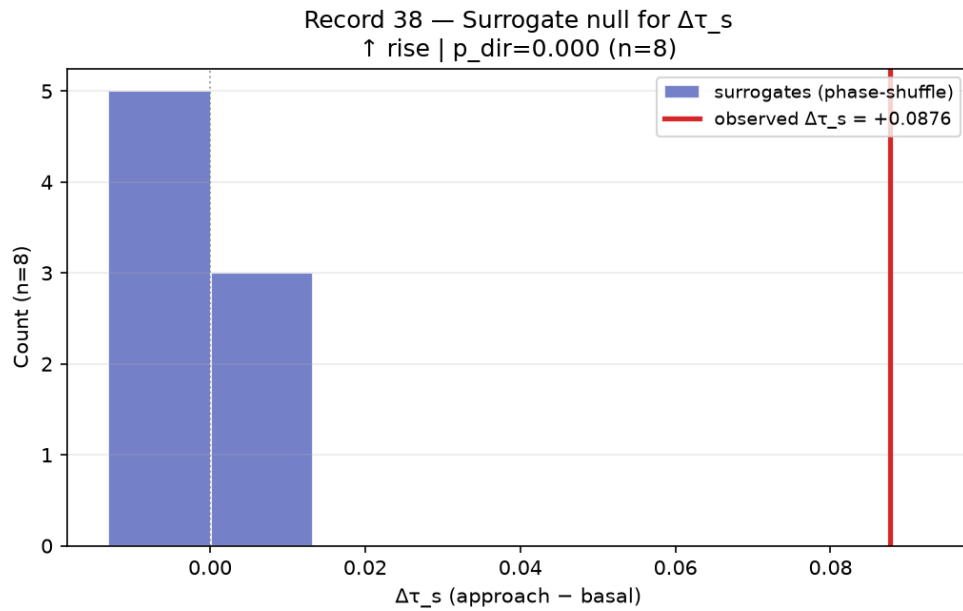


Figure 7: Phase-shuffle surrogate distribution for $\Delta\tau_s$ (record 38). The observed delta is extreme relative to surrogates that destroy cross-dependence.

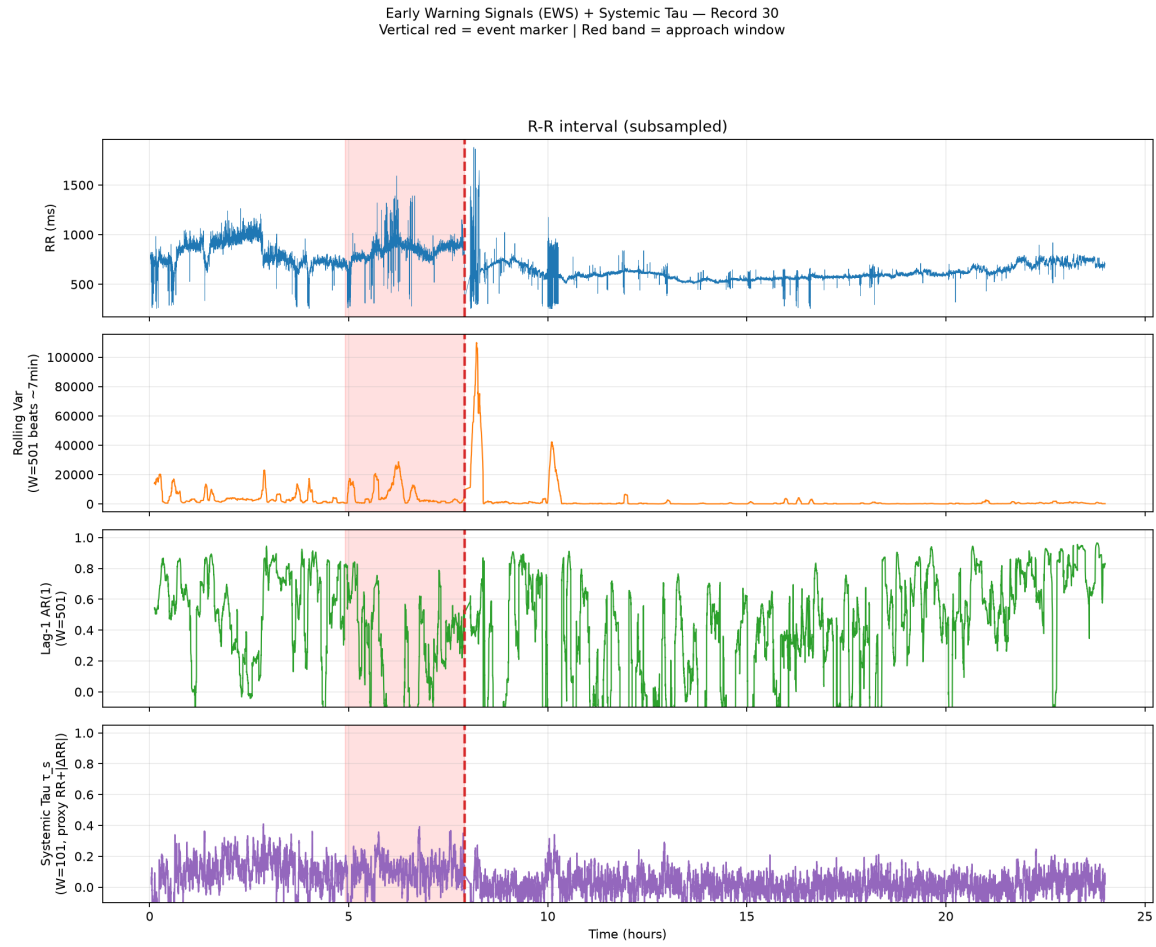


Figure 8: EWS panels for record 30: variance can rise while τ_s shifts negatively—illustrating dissociation between univariate amplitude and relational reorganization.

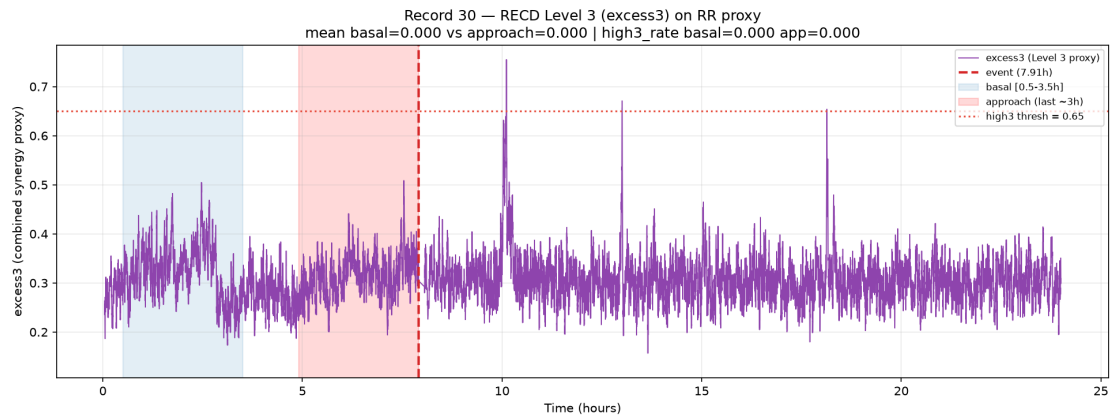


Figure 9: Excess3 dynamics for record 30 (negative $\Delta\text{excess3}$, concordant with $\Delta\tau_s$).

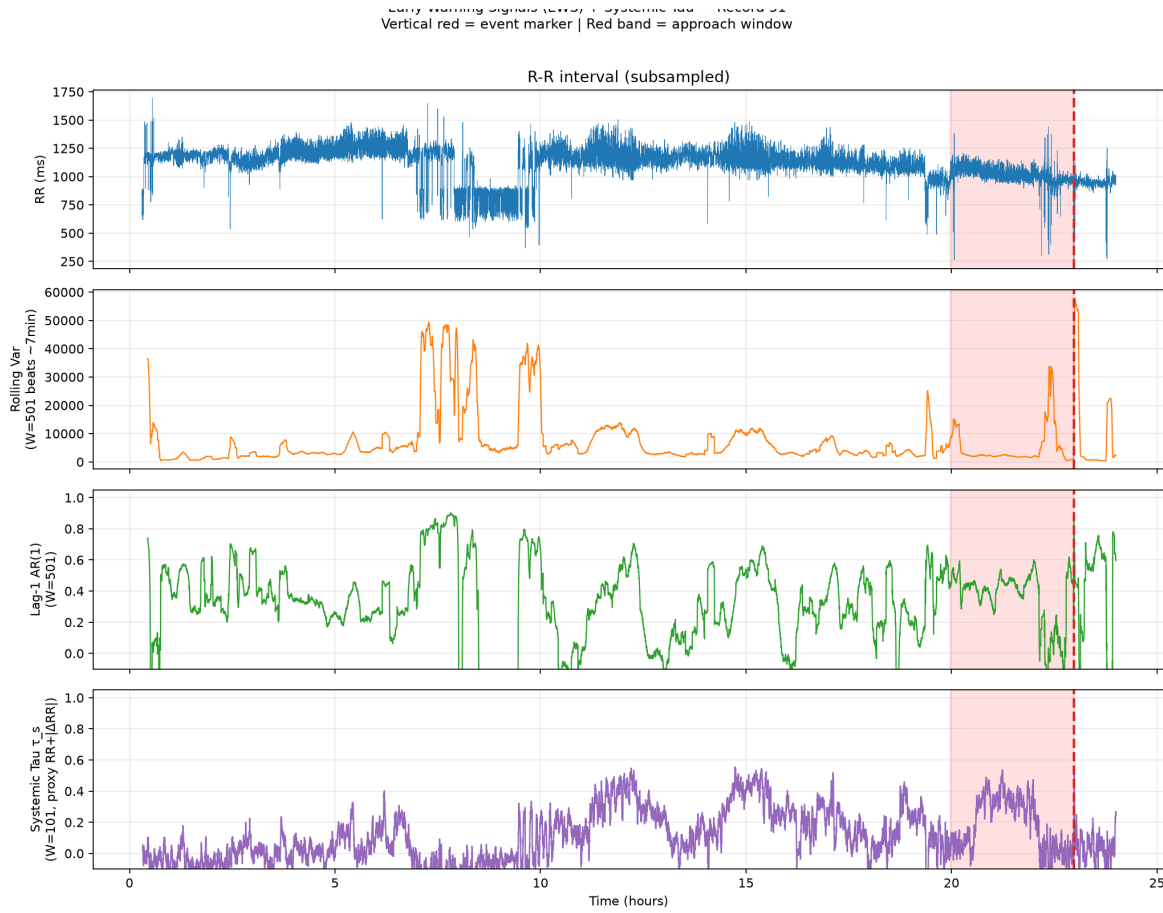


Figure 10: EWS panels for record 51 (intermittent pacing flagged). Relational deltas remain large and sign-concordant.

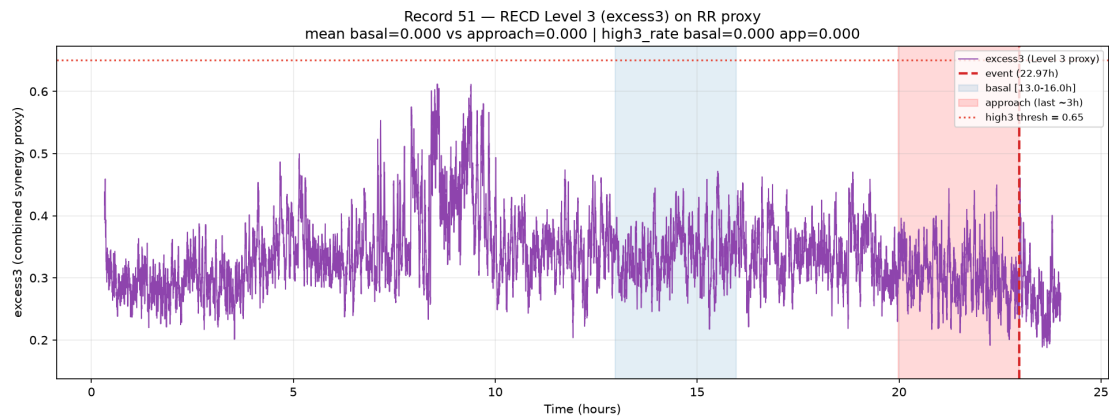


Figure 11: Excess3 for record 51.

Record 38 (strong positive $\Delta\tau_s$ and Δ_{excess3})

Record 30 (negative relational shift with classical variance increase)

Record 51 (intermittent pacing retained; strong negative concordance)

3.3 Light re-calibration and the status of high-level rate

Observed on the 10 RR series: median $|\Delta\tau_s| \approx 0.026$ (max ≈ 0.088); basal excess3 typically ≈ 0.30 – 0.35 , with approach values up to ≈ 0.43 on the strongest records.

The binary high-level3 rate remained zero at the recalibrated threshold (0.65) because observed excess3 values in these noisy RR series ranged between ~ 0.30 (basal) and ~ 0.43 (approach). Consequently, the continuous mean excess3 and its delta are used as the primary metrics; the rate metric is retained for future use on cleaner or longer recordings.

Deltas for mean excess3 are nearly unchanged under θ_3 adjustment (raw excess3 is independent of θ_3 at the reporting stage). When both weighted and unweighted RECD exist, deltas are numerically almost identical (e.g., record 38: unweighted $\Delta \approx 0.04151$, weighted $\Delta \approx 0.04136$), consistent with nearly constant λ under small $|\tau_s|$.

3.4 Classical EWS versus relational metrics

Across the cohort:

- **Variance** almost always increases from basal to approach (classical CSD-compatible amplitude rise).
- **Lag-1 autocorrelation** frequently *decreases*—opposite to naive CSD.
- τ_s and excess3 capture the *direction of relational reorganization*, which can align with or oppose the variance story depending on record context (terminal vs intermediate event geometry; AF substrate; pacing).

This dissociation is not a failure of EWS theory; it is evidence that pre-VF Holter dynamics are not a single-parameter approach to a fold bifurcation. Relational metrics add a complementary axis of description (Schaffer et al. 2009; Ivanov 2021).

3.5 Weighted RECD

Weighted RECD ($\alpha(\lambda)$ driven by $|\tau_s|$) completed on 6/10 records. Because empirical $|\tau_s|$ is far below synthetic activation scales, $\lambda(t)$ is nearly flat and frac-contrib3 remains high and relatively stable. The continuous excess3 delta—not the binary high-level rate—carries the differentiating signal. Full λ activation is expected to matter more after further threshold work on cleaner multichannel recordings.

4. Discussion

4.1 Context-dependent reorganization as the central message

The strongest interpretive claim supported by these data is not “a universal numeric threshold predicts VF,” but rather: **the relational organization of heart-rate dynamics reconfigures before spontaneous VF, and the sign of that reconfiguration is itself informative.** Positive $\Delta\tau_s/\Delta\text{excess3}$ and negative $\Delta\tau_s/\Delta\text{excess3}$ both appear, each with surrogate support in strong cases. This context dependence is a feature of physiological diversity, not noise to be averaged away.

For readers new to Systemic Tau, a useful analogy is the difference between measuring the loudness of a conversation (variance) and measuring whether the speakers are coordinating, interrupting, or fragmenting their dialogue (relational structure). Both can change before a social “critical event”; only the second tracks reorganization of interaction.

4.2 Relation to classical EWS and network physiology

Critical-slowness indicators remain theoretically important for systems near certain bifurcations (Scheffer et al. 2009; Dakos et al. 2012). Cardiac RR series under Holter conditions, however, combine non-stationarity, ectopy, AF, pacing, and autonomic modulation (Ivanov et al. 1999; Bashan et al. 2012; Bartsch et al. 2015). Network Physiology emphasizes that organism-level states emerge from coordinated organ-system interactions (Bashan et al. 2012; Ivanov 2021). Our bivariate $RR-|\Delta RR|$ proxy is a deliberately minimal relational embedding of that idea at the heart-rate scale: level and successive irregularity as coupled observables.

Prior SDDb studies have focused on classification accuracy for SCD/VF prediction using features and machine learning (Velázquez-González et al. 2021; Heng et al. 2020). The present work is complementary: it asks a dynamical-systems question about pre-event reorganization and reports transparent effect directions, surrogates, and quality flags rather than a single black-box score.

4.3 Systemic Tau / RECD in the broader research program

Systemic Tau and RECD were developed as general tools for ordinal early-warning analysis in complex systems, with theoretical synthesis and software releases documented in the author’s prior work (Padilla-Villanueva 2025b, 2026a, 2026b, 2025a). Applications to public-health surveillance (e.g., dengue early warning) and to chaotic modular systems motivate the same design choices used here: ordinal robustness, multivariate coupling, and hierarchical symbolic accumulation. The Holter results supply a high-stakes physiological testbed in which classical variance-based signals are known to be incomplete.

4.4 Pacing, AF, and borderline cases

Intermittent pacing and AF were not excluded a priori. Automated flags and interpolation fractions accompany every claim. Record 32 (intermittent pacing, interpolation fraction 5.22%) was retained after automated

quality flagging but exhibited noisier dynamics in the approach window; its inclusion did not reverse the overall sign-concordance pattern. Records 47 and 50 illustrate that small absolute deltas can break concordance—a desirable honesty condition for any early-warning metric.

5. Limitations

1. **Scale of SDDb.** The database contains only 23 records. After strict filters, $N \approx 8\text{--}12$ is the realistic high-quality ceiling; here $N = 10$. This scale is appropriate for a first relational study and for complex-systems / network-physiology venues, but not for multi-center clinical claim-making.
2. **Annotation heterogeneity.** Audited `.atr` annotations are preferred; unaudited `.ari` fallbacks are required for some records. Pacing detection is robust (comment + known list + CV heuristic) but paced RR interpretation remains cautious.
3. **Sparse clinical metadata.** Age, sex, medications, ejection fraction, and etiology are often missing or incomplete. Subgroup clinical interpretation is therefore limited.
4. **Single retrospective public corpus.** Recordings originate primarily from 1980s Boston-area hospitals. External validation on independent pre-VF Holter collections is mandatory before translation.
5. **Small absolute magnitudes.** Physiological $|\tau_s|$ and excess3 deltas are modest; synthetic-derived thresholds required light re-calibration. The binary high-level rate remains uninformative at 0.65 on these series.
6. **Surrogate budget.** Phase-shuffle ensembles use $n = 8$ per record (light but directional). Larger ensembles and alternative surrogate classes (IAAFT, twin surrogates) are natural extensions.
7. **Bivariate proxy.** The $\text{RR} - |\Delta\text{RR}|$ embedding is intentional and minimal; richer multichannel ECG or multimodal Network Physiology embeddings may strengthen λ -weighted RECD.

6. Conclusions

Systemic Tau combined with ordinal RECD provides a reproducible, sign-concordant early-warning signature of spontaneous VF that is invisible or reversed by classical univariate EWS in several records. The relational metrics detect reorganization even when variance-based signals are weak or reversed; intermittent pacing and AF, when explicitly flagged, do not abolish the pattern.

These findings provide real-world physiological support for the Systemic Tau and RECD framework as tools capable of detecting context-dependent relational reorganizations that precede critical transitions, even in noisy biological signals where classical univariate early-warning signals fail or reverse.

Next steps are (i) external validation on independent Holter VF collections, (ii) richer multivariate proxies under the Network Physiology program, and (iii) prospective evaluation of online ordinal monitors for research (not clinical) early-warning dashboards.

Data and Code Availability

- **SDDB:** PhysioNet Sudden Cardiac Death Holter Database, DOI [10.13026/C2W306](https://doi.org/10.13026/C2W306) (PhysioNet 2004; Greenwald 1986; Goldberger et al. 2000).
- **Systemic Tau software:** systemictau Python package and related releases (Padilla-Villanueva 2026b), <https://github.com/johelpadilla/systemictau>.
- **Analysis repository:** project scripts and batch outputs for this study (RR extraction, batch runner, RECD modules, figure generation) are distributed with the preprint package.
- **Theory:** prior Systemic Tau / RECD preprints and syntheses (Padilla-Villanueva 2025b, 2026a, 2025a).

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Author Contributions

J.P.-V. conceived the study, developed the Systemic Tau / RECD framework, implemented the analysis pipeline, performed the SDDB experiments, prepared figures, and wrote the manuscript.

Competing Interests

The author declares no competing financial interests. The systemictau software is released under an MIT license; theoretical manuscripts are under CC-BY where indicated on Zenodo / Preprints.org.

Ethical Statement

This study analyzes exclusively de-identified, publicly available Holter recordings from PhysioNet SDDB. No new human-subjects enrollment was performed.

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Appendix A. Reproducibility commands

```
cd Investigaciones/Cardiac_CCTP_Pilot
```

```
# Full batch with final parameters
```

```
python3 code/run_cctp_batch.py \
  --records 30,31,32,35,36,38,45,47,50,51 \
  --theta3 0.08 --high-thresh 0.65 --lambda-relative --force
```

```
# Single-record RECD (example: strongest positive case)
```

```
python3 code/run_recd_on_rr.py --record 38 --theta3 0.08 --high-thresh 0.65
python3 code/run_recd_weighted_on_rr.py --record 38 \
  --theta3 0.08 --high-thresh 0.65 --lambda-relative
```

```
# Inspect consolidated table
```

```
python3 -c "
import pandas as pd
df = pd.read_csv('results/cctp_batch_summary.csv')
cols = ['record', 'delta_tau', 'p_tau_surrogate', 'delta_excess3',
        'p_excess3', 'interp_frac', 'pacing_detected', 'has_weighted']
```

```
print(df[cols].to_string(index=False))
"
```

Appendix B. Notation

Symbol	Meaning
τ_s	Systemic Tau (windowed ordinal/relational coupling metric)
$\Delta\tau_s$	Approach minus basal mean τ_s
Φ_1, Φ_2, Φ_3	Nested ordinal RECD levels
excess3	Continuous surplus contribution of highest RECD level
θ_3	Threshold parameter for level-3 logic (re-calibrated to 0.08)
λ	Weighting factor derived from $ \tau_s $ (relative scaling used)
W_τ, w_ϕ	Sliding windows for τ_s and ordinal RECD (101 beats)
m	Bandt–Pompe embedding dimension (3)

Supplementary Information (for journal upload)

The following materials accompany the main text and are included in the preprint package:

S1. Batch figures (main). batch_delta_tau_excess3.png, batch_significance.png, batch_quality_interp.png.

S2. Per-record diagnostic panels. For key records 30, 35, 38, 50, 51: EWS multi-panels (06_ews_panels.png), excess3 traces and boxes (16, 17), surrogate histograms (15), and, where available, weighted RECD contributions (18, 19).

S3. Sensitivity note on re-calibration. Changing θ_3 from 0.10 to 0.08 and high-threshold from 1.75 to 0.65 does not reverse sign concordance; continuous $\Delta\text{excess3}$ remains the primary readout. Relative λ is recommended when $|\tau_s| \ll \text{synthetic design scale}$.

S4. Quality inventory. Per-record n_beats, interp_frac, cv_rr, pacing_detected, known_pacing_type, and has_weighted columns in cctp_batch_summary.csv.

S5. Extended discussion of borderline records 47 and 50. Small absolute $\Delta\tau_s$ with non-concordant $\Delta\text{excess3}$ should be reported as indeterminate / borderline rather than forced into a binary “alert” narrative.

S6. Software versions. Analyses used systemictau (see Zenodo/GitHub release notes contemporaneous with this preprint), WFDB, and standard scientific Python libraries.

Manuscript version: journal-ready v1.0 (July 2026).

Corresponding author: Johel Padilla-Villanueva, DrPH — University of Puerto Rico, Medical Sciences Campus — ORCID 0000-0002-5797-6931.